WHO EMERGENCY UNIT FORM: GENERAL

☐ Mass Casualty

Hospital Registration Number:

Date: DD/MM/YY

Time of Arrival:__ __: __ __(24h)

Patient Surname:
First Name:

Age: _____
INF / CH / AD

Sex: ☐Male ☐Female
☐ Other: _____

Date of Birth:
DD/MM/YY

Weight: kg

Occupation: ☐ Unknown
Patient Residence (at least City and Sub-district): ☐ Unknown
Patient defined racial and ethnic identity: ☐ Unknown
Is an interpreter required? ☐ Yes ☐ No ☐ Not answered

Arrival Mode:
☐Ambulance ***Level of prehospital provider:*** ☐ Basic ☐ Advanced
Emergency system activation Date: DD/MM/YY Time: ___:___ (24h)
Emergency system dispatch Date: DD/MM/YY Time: ___:___ (24h)
Emergency personnel arrival on scene Date: DD/MM/YY Time: ___:___(24h)
Emergency personnel arrival on scene Date: DD/MM/YY Time: ___:___(24h)
☐Car/Truck (*circle* Private or Taxi)
☐ Motorized 2/3-wheeler (*circle* Private or Taxi)
☐ Public Transport ☐ Walk ☐ Other: _____

Contact Person:
Phone:
Relation:

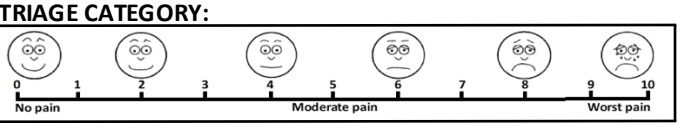
Number of prior facilities: ___
Referred from:

☐Ambulatory Non-Ambulatory: ☐ Acute ☐ Chronic
Day to day activities limited by health problem or disability including old age? ☐ Yes ☐ No

CHIEF COMPLAINT:

INITIAL VS at ___ ___: ___ ___(24h)
Temp:____ BP:____ / ____ Pulse:_____ RR:____
SpO_2:____ % on _____ Pain score: ____ / 10

TRIAGE CATEGORY:



TREATING PROVIDER ASSESSMENT: Date: DD/MM/YY **Time** ___ ___: ___ ___(24h) ☐ **Dead on arrival**

HIGH RISK SIGNS

☐ Abnormal AVPU ☐ HR <55 or >130 (adult) ☐ Temp >39˚C or <36˚C ☐ SpO_2 <90% on RA
☐ Stridor, voice change or unable to swallow ☐ Respiratory distress (grunting in child, retractions, cyanosis)
☐ Poor perfusion, weak pulse, capillary refill >3s ☐ Vomits everything, can't drink or feed

PRIMARY SURVEY: (see Reference Card for normal findings, only mark NML if all key elements are normal)

Airway
☐ NML

☐ Angioedema ☐ Stridor ☐ Voice changes
☐ Oral/Airway burns
Obstructed by: ☐ Tongue ☐ Blood ☐ Secretions
☐ Vomit ☐ Foreign body

Airway: ☐ Repositioning ☐ Suction ☐ OPA ☐ NPA ☐ LMA
☐ BVM ☐ ETT

Breathing
☐ NML

Spontaneous Respiratory Rate: _____
Chest Rise: ☐ Shallow ☐ Retractions ☐ Paradoxical
Trachea: ☐ Midline ☐Deviated to ☐L ☐R
Breath Sounds: ☐ L_____ ☐ R_____

Oxygen: _____ L
☐NC ☐Mask ☐NRB
☐BVM ☐CPAP/BIPAP
☐Ventilator
☐Bronchodilator

Chest needle or tube (circle):
☐ L – Size: _____ Depth: _____ cm
☐ R – Size: _____ Depth: _____ cm

Circulation
☐ NML

Skin: ☐ Warm ☐ Dry
☐ Pale ☐ Cyanotic ☐ Moist ☐ Cool
Capillary refill: ☐ <3 sec or _____ sec
Pulses: ☐ Weak ☐ Asymmetric_____
JVD: ☐ Yes ☐No

Access: ☐IV: Loc _____ Size _____
☐CVL: Loc _____ Size _____ ☐IO: Loc _____ Size _____
☐IVF:_____mLs ☐NS ☐LR ☐Other _____
☐Blood ordered ☐Epinephrine given

Disability
☐ NML

☐ A ☐ V ☐ P ☐ U
☐ **Moves all extremities** or ☐ Deficit:
_____
Pupils: Size: L _____ R _____
Reactivity: L_____ R_____

Blood Glucose:

(Abnormal if < 3.5 mmol/dl)

☐ Glucose ☐ Naloxone
☐ Antiepileptic
☐ Others:

HISTORY OF PRESENT ILLNESS:

(Symptoms, time course, exacerbating and alleviating factors, prior episodes & prior interventions, including any primary health care)

REVIEW OF SYSTEMS: (See Reference Card for normal findings. Do NOT mark normal unless all key elements are normal.)

☐ NML	General:	☐ NML	Reproductive:
☐ NML	HEENT:	☐ NML	Skin:
☐ NML	Resp:	☐ NML	MSK:
☐ NML	CV:	☐ NML	Heme:
☐ NML	GI:	☐ NML	Neuro:
☐ NML	Pelvis/GU/Rectal:	☐ NML	Psychiatric:

Form to be used with WHO Reference Card. See who.int/emergencycare for more information.

PAST MEDICAL HISTORY: History obtained from:

Medications: □ Unknown

Past Medical: □ HTN □ DM □ COPD □ Psych □ Renal Disease □ Unknown
Other:

Past Surgeries (type & date)**:** □ Unknown

Allergies: □ Unknown

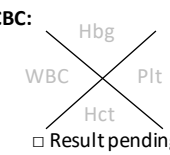
Last Menstrual Cycle: _____ **G**____**P**____ □ Unknown
Pregnant? (circle) Yes / No □ Reported □ Testing done
Vaccinations up to date? □ Unknown □No □Yes _____
Substance Use: □ Tobacco □ Alcohol □ Drugs □ IV Drugs □ Unknown
Family History: _____
Safe at home? _____

PHYSICAL EXAM: (See Reference Card for normal findings. Do NOT mark NML unless all key elements are normal. Specify L or R if needed.)

□NML	**General**		□NML	**Cardiac**	
□NML	**Neuro/Psych**		□NML	**Abdominal**	
□NML	**HEENT**		□NML	**Pelvis/GU/ Rectal**	
□NML	**Neck**		□NML	**Lymph**	
□NML	**Respiratory**		□NML	**MSK**	
			□NML	**Skin**	

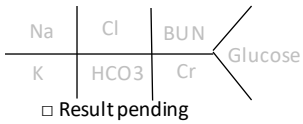
DIAGNOSTICS: (Labs, Imaging)

CBC:



□ Result pending

Lytes/Cr/glucose:



□ Result pending

Urine Dip: Glu: Ket: Blood:
Nitr: Leuk: Prot:

UPT: □ Pos □Neg
Malaria: □ Pos □Neg
HIV Rapid: □ Pos □Neg
Blood type: _____

ECG: Rate: _____
Sinus rhythm? □ Y □ N
Ischemia? □ Y □ N
Interpretation:

Other labs/imaging:

ADDITIONAL INTERVENTIONS:

Fluids and Medications Given	**Time** (24h)
□ IVF: _____ mLs □NS □LR □Other _____	____:____
□ Blood products (specify number of units given):	____:____
_____	____:____
□ Opioid Analgesia:_____	____:____
□ Other Analgesia:_____	____:____
□ Sedation/Paralytics:_____	____:____
□ Antimicrobials:_____	____:____
□ Tetanus: _____	____:____
□ Other:	

Procedures (include time and outcome)	**Time** (24h)
□ Intubation: _____	____:____
□ Chest Tube: _____	____:____
□ Lumbar Puncture: _____	____:____
□ Simple / Complex Laceration Repair: _____	____:____
□ Other:	

ASSESSMENT (include summary and differential) AND PLAN (imaging; meds/interventions; consults with time called/arrived and recs):

REASSESSMENT at ___ ___: ___ ___(24h) □ **Condition same**
Temp:_____ Pulse:_____ BP:____ / ____ RR:_____ SpO_2:____ % on_____ Changes:_____

DISPOSITION: Checklist completed: □ Y □ N ED departure (date & time): DD/MM/YY ___ ___ : ___ ___ (24h)

Diagnoses/Impressions (list all)**:**

□ **Admit to:** □ Ward _____ □ ICU □ OT
□ **Discharge:** Plan discussed with patient? □ Yes □ No
□ **Transfer to**: _____
□ **Left without being seen or before treatment complete**
□ **Died of** (specify cause - NOT cardiopulmonary arrest):
_____

VS at Dispo at: ___ ___: ___ ___ (24h)
Temp:____ Pulse:____ BP:____ / ____ RR:____ SpO_2:___ % on_____

Accepting Provider: _____

Emergency Unit Provider Name/Title (include handovers)	**Signature and Date**

Chart to be used with WHO Reference Card. See who.int/emergencycare for more information.